

Case 1

- An otherwise healthy 8-year-old female presents for evaluation of a dome-shaped red-brown papule on the face that appeared 4 months ago and has grown rapidly. It is asymptomatic, and has not bled.



Spitz Nevus

- Round, well-circumscribed, dome-shaped, smooth-topped, hairless, small (<1 cm) papules or nodules
- Most often pink, tan, brown, sometimes black
- Often a history of **recent rapid growth** (arise within months)
- Most common location is the head and neck
- occur at all ages
 - 1/3 of patients are children
 - rarely seen > 40
- Histologic examination must be done to confirm the clinical diagnosis.
 - Spitz nevi are benign, but there can be a histologic similarity to melanoma.
 - Years ago these were called "juvenile melanoma."

Halo Nevus

- History: 3 stages
 - Development of a halo around a preexisting nevus, sometimes the halo may be preceded by faint erythema (redness)
 - Disappearance (months to years) of the nevus
 - Repigmentation (months to years) of the halo



Blue Nevus

- Acquired, asymptomatic, benign
- PE: 10mm, firm, round to oval, dark-blue to gray-black, sharply-defined papule
- commonly found on the dorsa of hands or feet, buttocks, back, scalp or face



Nevus Spilus

- light brown pigmented macule from a few cm to >15cm
- the distinctive feature of this lesion is the many dark brown small macules or papules (2mm) scattered throughout the pigmented background



Seborrheic Keratosis

- The most common of the benign epithelial tumors.
- hereditary, begin to appear around age 30 and continue to occur over a lifetime, varying from a few to hundreds in some very elderly patients.
- Lesions are benign and do not require treatment except for cosmetic reasons.

Case 2a

- Pt states all of these lesions appeared abruptly over the last month. What is this called and why are you concerned?
- Sign of Leser-Trelat
- Internal malignancy (most commonly GI)

Case 3

1. 33 yo female with erythematous, slightly hyperpigmented dermal nodule on the leg
2. occurred after shaving



Dermatofibroma

- Very common
- Fibrous reaction to trauma or bite
- Legs>arms>trunk
- Important only because of its cosmetic appearance or its being mistaken for other lesions, such as malignant melanoma when it is pigmented.
- Usually asymptomatic papule or nodule, 3 to 10 mm in diameter.
- Firm, dome-shaped, or may be depressed
- Color is variable—skin-colored, pink, brown, tan, dark chocolate brown
- *Dimple sign*: lateral compression with thumb and index finger produces a depression or "dimple"

Case 4

- 64 yo obese female presents with these flesh colored pedunculated papules on her posterior and lateral neck that become irritated when she wears a necklace.
- She also has some under her armpits, breasts



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Case 5

- 24 yo construction worker presents with this lesion that developed quickly after he injured his finger at work.
- Pt states it bleeds frequently



Pyogenic Granuloma

- Rapidly developing hemangioma arising at sites of minor trauma
- Bright red to violaceous, smooth, dome-shaped nodule
- bleeds spontaneously or after minor trauma
- Fingers, lips, mouth, trunk, toes
- Treatment is surgical excision with electrodesiccation at the base.
- The importance of **pyogenic granuloma** is that it can be mistaken for amelanotic nodular melanoma, and vice versa.

Pyogenic Granuloma



- What else is in your differential diagnosis for this lesion on the face of a child?

Spitz nevus

Case 6

- A 50 year old male presents with flesh to yellow colored papules on the cheeks
- They have a donut shaped appearance
- They are not painful but he is getting more and more of them over time
- they affect him cosmetically

What's your differential diagnosis?

syringomas

- Sweat duct tumor-
benign adenoma
- 1-2mm flesh colored
papules
- Multiple, **lower lids**,
face, axilla, umbilicus,
chest, vulva



What's your differential diagnosis?

trichoepithelioma

- Benign appendage tumors
- Occur at puberty
- Mc on face
- Pink to skin colored papules
- May appear similar to basal cell Ca



What's your differential diagnosis?

Milium

- 1-2 mm superficial white to yellow, keratin-containing epidermal cyst
- located on eyelids, cheek, forehead, & sites of trauma
- Occur at any age
- Tx: incision & expression of contents



Case 7

- A baby presents with a red strawberry appearing plaque on her trunk
- The parents are concerned because it is rapidly growing
- the dermatologist tells them not to worry as the lesion may disappear before she is 10 yo

What's your differential diagnosis?

Vascular malformations

Cavernous hemangioma

- deep, vascular malformation composed of capillary, lymphatic, & venous tissue
- soft, compressible blue-tinged, erythematous nodules



Port Wine Stain

- Vascular capillary malformations **that do not** undergo spontaneous involution
- irregularly shaped red to violaceous patch present at birth
- do not cross midline; dermatomal
- Usually confined to skin but may affect eye and leptomeninges



What's your differential diagnosis?

Cherry Angiomas

- common, asymptomatic, bright red domed papules
- principally on trunk
- first occur at 30 yrs & increase in number over years



Actinic Keratosis

- Common, sun-induced premalignant lesions
- Single, or multiple, discrete, rough, adherent, scaly papules
- Arise in areas of dermatoheliosis (head, neck, hands, sun damaged areas)
- Premalignant potential, may develop into SCC (squamous cell carcinoma), cutaneous horn
- Tx: cryotherapy, topical 5-fluorouracil (5-FU)

Basal Cell Carcinoma

- Most common type of skin cancer
- locally invasive & aggressive; limited capacity to metastasize
- Occur over age 40, pts w/poor tanning capacity & albinos
- 7 clinical types: **superficial, nodular, pigmented, ulcerative** (“rodent ulcer”), **sclerosing, cystic, basosquamous**
- Occurs on head, neck, face, canthi, nasolabial

BCC ulcerating

A large scar-like lesion, containing several small nodules of BCCs. On the lateral canthus of the eye there is a large ulcer with rolled borders representing rodent ulcer.



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Superficial BCC- red scaly plaque, pearly white border, resembles eczema, psoriasis, least aggressive



Case 10

Describe this lesion
on the upper back of
a 54 yr old female

Lesion has been there
for 4 mos and didn't
respond to treatment
with topical
Hydrocortisone



A

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SCCIS (squamous cell carcinoma in situ)

Bowen's disease

- sharply demarcated, pink to red, scaling, papules, or plaques.
- SCCIS is most often caused by UVR (ultraviolet radiation) or HPV (human papilloma virus) infection.
- Commonly arises in lesions such as solar keratoses or HPV-induced squamous epithelial lesions
- Similar but usually nonscaly lesions on the glans and vulva are called *erythroplasia*.
- Anogenital HPV-induced SCCIS is referred to as *bowenoid papulosis*.
- Untreated SSCIS may progress to invasive SCC.
- Treatment: 5-fluorouracil, imiquimod, cryosurgery, CO₂ laser, or excision, including Mohs micrographic surgery.

Case 11

Describe these lesions



Invasive Squamous Cell Carcinoma

- Malignant tumor of keratinocytes
- SCC usually arises in epidermal precancerous lesions and, depending on etiology and level of differentiation, varies in its aggressiveness.
- Age of onset: > 55 yrs in U.S.
- indurated erythematous, hyperkeratotic, scaly papules, plaques, nodules, +/- erosions
- Occur on face, tips of ears, preauricular area, scalp, dorsal hands, forearms, lower lips
- Tx: cryotherapy, ED&C (debride and curette), Mohs, surgery
- The majority of UVR-induced lesions have a low rate of distant metastasis in otherwise healthy individuals. More aggressive SCCs occur in immunosuppressed pts

Case 12

- 55 yo healthy male
- 1 cm lesion on his back
- His wife wants you to biopsy it
- It has been present since birth
- He doesn't believe it has changed in appearance



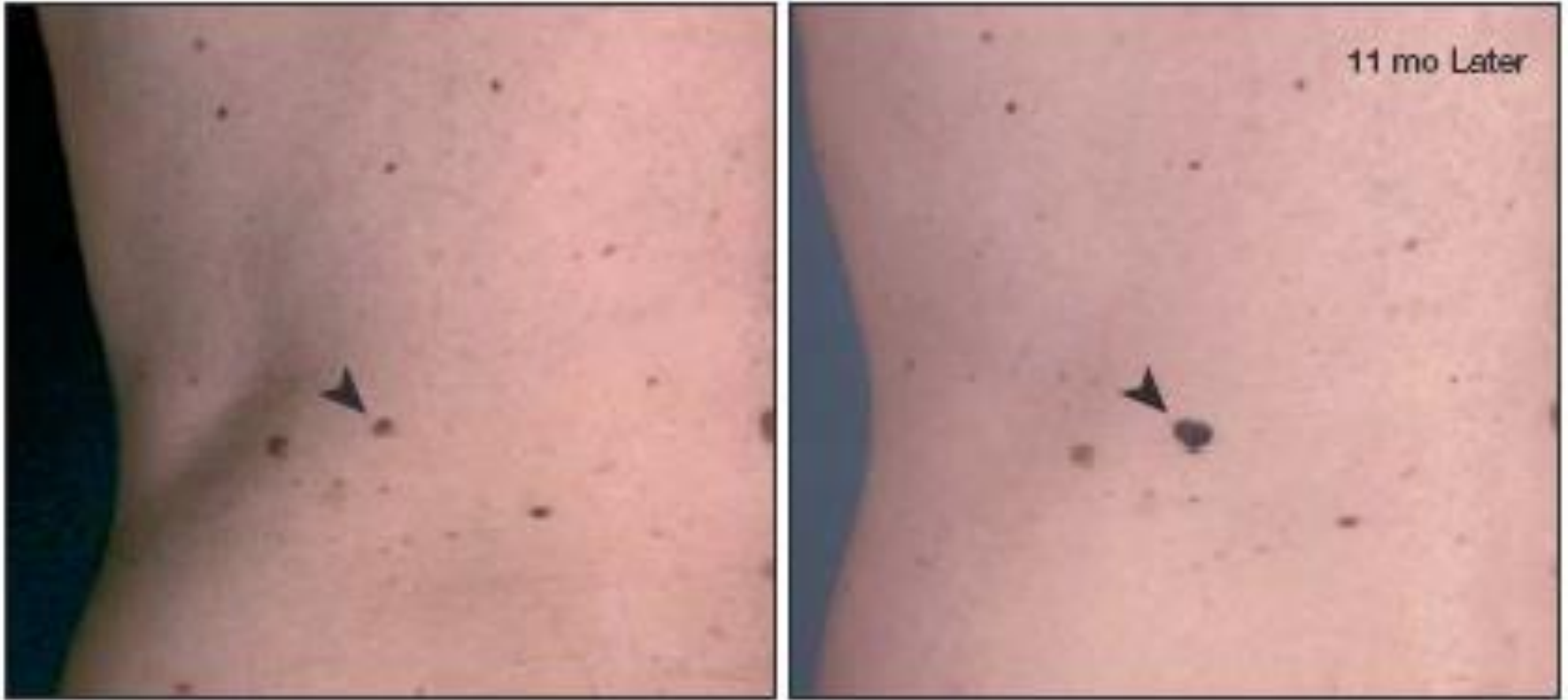
Congenital Nevus

- Mole present since birth
 - Highly pigmented
 - Hair growth in lesion
- Small < 1.5 cm, Medium 1.5-20 cm
 - Lifetime risk of melanoma equal to general population
- Giant nevi > 20 cm
 - Estimated lifetime risk of melanoma 3-8%
 - Prophylactic removal may be indicated
 - 40% of melanomas in kids develop within these nevi



Case 13

28 yo female presents with this lesion:



What next?

Acral Lentiginous Melanoma

- Incidence equal in Caucasian & dark-skinned pts
- 70% of melanomas in African-Americans, Latinos, Asians
- Location of lesions
 - palms, soles, terminal phalanges, nail beds
 - mucous membranes
- poor prognosis
 - More advanced at time of diagnosis
 - Tend to metastasize early

